



Introduction to Internal Family Systems (IFS)

Richard C. Schwartz, who has a Ph.D. in marital and family therapy, and is one of the authors of this manual, developed internal family systems therapy (IFS) in the 1980's while treating eating disordered (ED) adolescents who routinely spoke about internal conversations with what they called "different parts." Following their vernacular lead, he referred to their subpersonalities as "parts." While exploring options and encouraging the clients to interact with their ED parts the way members of a family interact, Schwartz learned that he and the client could persuade an extreme ED part to permit the client some mental separation from its distorted perspective, whereupon the client would spontaneously become mindful (nonjudgmental and curious) toward the part.

This kindly – ultimately compassionate – relational stance between client and parts proved crucial to healing and became the linchpin of IFS therapy. We've all had moments of clarity and balance when the incessant chatter inside our head ceases, we feel calm and spacious, as if our mind, heart and soul had brightened and expanded. At other times, we feel a wave of joyful connection with others that washes away irritation, distrust, and boredom. Schwartz observed that healing just happens when therapist and client achieve a critical mass of this phenomenon, which he dubbed the *Self*.

EMBODY THE SELF AND LISTEN TO PARTS

The goal of IFS, now an evidence-based treatment, is to embody the Self and heal our injured parts so that we can live with confidence, guided by curiosity and compassion. As we illustrate throughout this manual, when clients embody more Self and listen to their parts rather than trying to eliminate these aspects of themselves, their inner dialogues change spontaneously. Their extreme voices calm down, they begin to feel good things: safer, lighter, freer, more open, more playful. Clients who've shown little insight into their problems are suddenly able to trace the trajectory of their own feelings and emotional histories with clarity and understanding. And even very disturbed clients who appear to be unlikely candidates for such shifts after living

through childhoods of relentless abuse and neglect will experience insight, self-acceptance, stability and personal growth once they connect with the Self at their core.

After repeatedly witnessing a sudden acquisition of this ability to contain and understand even severe inner turmoil, Schwartz concluded that the traditional trajectory of therapy (symptom-oriented, results-focused, problem solving) did not encompass, and could not help him understand, what he was tapping into. Psychotherapy and spirituality alike describe the “essence” that we call the Self with terms like soul, the divine, Buddha nature, or the core seat of our consciousness. In his experience, once parts make space we can all have access to this core of who we really are.

Yet being Self-led with clients isn’t easy. So much of what we’re taught about the psyche and therapy fuels our fears and keeps us distant. The DSM in all its iterations has always encouraged us to focus on our clients’ scariest and most pathological behaviors, while concerns for our careers, reputations and the potential of litigation encourage us to be on guard. At the same time, since our clients stir up as many unruly feelings, thoughts, prejudices, negative associations, and untoward impulses in us as we stir up in them, our personal baggage makes us vulnerable in sessions and we cannot help our clients until we can help ourselves. We cannot go with them into their terror, humiliation and devastating loneliness unless we’ve explored ours first.

As a result, we can’t omit and ignore our inner barbarians, those unwelcome parts who hate, rage, suppress, terrorize, betray, threaten and engage in all manner of prejudice and greed – or those who have somewhat less abhorrent feelings like depression, anxiety, self-righteousness, guilt and selfloathing. Nevertheless, once we have befriended our own extreme reactions - for befriending is a more beneficial, popular and effective activity than berating - the job of helping clients befriend theirs offers significant benefits for us therapists. When we listen to our parts rather than exiling them, we don’t have to work so hard and the parts are able to transform.

To attune with our clients we must make a big investment in ourselves, which can be a challenge. But once we are able to connect internally, sessions flow with an almost effortless quality, as if something magical were unfolding. As IFS therapists, our job is to accompany our clients to a state of deep mindfulness, full-bodied attention, centered awareness, and inner calm. After a few hours in this energizing, enlivened state, having been privileged to witness the awe inspiring journeys our clients take inward, we often end the day feeling connected to something much bigger than we are.

HOW TO USE THIS BOOK

IFS is Experiential: Try Doing the Exercises in this Manual Yourself First

The best way to grasp the IFS approach to therapy is to have the experience. And the best way to practice IFS with clients is to know what it feels like personally. We recommend that readers engage with the exercises in this manual personally before using them with clients.

Meditations

This manual contains several meditations that are based on the IFS model. Feel free to use them in whatever way feels right. You may want to record them to play back, or you may be able to read them through and simply remember the steps. You may also want to go through them in sequence or you may want to try the same one several times before moving on to another.

Neuroscience

We have also incorporated some current knowledge on neuroscience as it relates to the steps of the IFS model of therapy. We hope this will enhance your understanding of what might be happening in the brain during an IFS session as well as inform your therapeutic decisions.

What a Manual on IFS Can Teach

In this manual we walk the reader through the flow of the IFS model of psychotherapy descriptively and experientially, introducing the reader to the positive motives, often hidden, that govern internal systems and illustrating effective strategies for addressing the problems that underlie symptoms.

We also illustrate the final healing steps of IFS therapy, which accompany the client into her or his greatest areas of vulnerability, but in this case we do not provide experiential exercises nor do we recommend that you try to do an unburdening. Instead we urge therapists who lack formal training in IFS to revert to their prior training and expertise at this point in therapy. To learn to practice IFS from the initial contact all the way through the final steps, and to maximize proficiency with the IFS model, we recommend the formal, experiential training offered by The Center for Self Leadership (CSL). We also recommend trying IFS therapy with an IFS therapist. For more information on the various training opportunities and a listing of IFS trained therapists, go to Selfleadership.org.

The IFS Model of Mind

IFS therapy uses a plural model of mind: we all have an internal system of countless parts who interact internally with each other and externally with other people. In addition, we all have a core resource that is not a part, which is characterized by balance, curiosity and compassion. Schwartz dubbed this nonpart resource “the Self.” Schwartz credits the systems focus of his family therapy training with helping him be open to conceptualizing the psyche of the individual as a system, which is consistent with what he was hearing from clients (Schwartz, 1995). In this inner system, some parts take on protective roles in response to relational injury, which is ubiquitous in childhood in one form or another and cannot be avoided. IFS attends to the needs of parts in both categories: protective and injured.

GLOSSARY OF IFS TERMS

Like all psychotherapies, IFS assigns unique meanings to certain words and phrases. Here is a glossary of the language in IFS.

5 P’s: The qualities of an IFS therapist: Presence, patience, persistence, perspective, and playfulness.

6 F’s: The steps we use to help protective parts differentiate from the Self. They include **F**ind, **F**ocus, **F**lesh-out, **F**eel, **b**eFriend and **F**ears.

8 C’s: The qualities of Self-energy: Curiosity, calm, clarity, connectedness, confidence, courage, creativity and compassion.

Blended (or undifferentiated): When a part is undifferentiated from another part or from the Self.

Burdened: When parts have taken on painful beliefs and feelings about themselves or have distressing physical sensations from external sources from which they cannot escape until they are unburdened.

Burdens: Negative, self-referential beliefs (*I’m unlovable, I’m worthless*) and intense, trauma-related feeling states (terror, shamefulness, rage), physical sensations or visions (flashbacks).

Direct Access: A method of communicating with parts, and an alternative to internal communication (a.k.a. in-sight). When a protector will not unblend, the therapist speaks directly

to the clients' parts. In direct access the therapist can speak *explicitly* to a part (e.g., "Can I talk to that part directly? Why do you want John to drink?"). Or, when the client rejects the idea of parts, and says, "That's not a part, that's me," the therapist can speak to it *implicitly*, without direct acknowledgement that she is talking to a part. Direct access is the usual method with children (see Krause, 2013), although some children are able to use internal communication.

Do-Over: When an exiled part takes the client's Self back into a time and place in the past where it is stuck and instructs the Self to do whatever the part needed someone to do for it at the time. After this, whenever the part is ready, the Self brings the part out of that scene into the present time.

Internal communication (a.k.a. in-sight): The primary approach used with adults to understand and communicate with parts, internal communication requires that the client be aware of parts (often aided by visual, kinesthetic, or aural experience) and have enough Self-energy to communicate with them directly. When protectors block internal communication we use direct access.

Parts: Internal entities, or subpersonalities, who function independently and have a full range of feelings, thoughts, beliefs, and sensations. These entities, who have their own Self-energy when they feel understood and appreciated, vary in appearance, age, gender, talent, and interest. They exist and take on various roles within the internal system. When not exiled or in conflict with each other over how to manage exiled parts, they contribute in a variety of ways to our efficient functioning and general well-being.

Three Types of Parts

IFS classifies parts in three broad categories according to how they function in relation to each other. An injured part, or *exile*, is primary in its influence on the behavior of other parts. Orbiting around exiles are two categories of protective parts. The proactive protector, called a *manager*, has the role of maintaining the individual's functioning despite what the exiles feel. The reactive protector, called a *firefighter*, has the role of distracting from and suppressing the emotional pain of exiled parts, which breaks through despite the best efforts of the manager.

1. **Exiles:** Revealed in feelings, beliefs, sensations and actions, these parts have been shamed, dismissed, abused or neglected in childhood and are subsequently banished by protectors for their own safety and to keep them from overwhelming the internal

system with emotional pain. A great deal of internal energy is expended to keep exiles out of awareness.

Protectors:

2. ***Proactive protectors or Managers:*** Proactive helpers who focus on learning, functioning, being prepared and stable. Managers are vigilant in trying to prevent exiles from being triggered and flooding the internal system with emotion. As a consequence, they are hard working and use a variety of tactics – not least, determined, relentless, criticizing and at times shaming – to keep us task-oriented and impervious to feelings.
3. ***Reactive protectors or Firefighters:*** Reactive protectors share the same goal as managers; they want to exile vulnerable parts and extinguish emotional pain. However, reactive protectors are emergency response workers. They get activated after the fact, when the memories and emotions of exiles break through despite the repressive efforts of managers. Reactive protectors tend to be fierce and use extreme measures that managers abhor, like alcohol and drug abuse, binge eating, excessive shopping, promiscuity, cutting, suicide and even homicide.

Polarization: An adversarial relationship between two protectors who are in conflict over how to manage an exile. Over time, their opposing views tend to become increasingly extreme and costly. However, when the intentions and contributions of each part are acknowledged by the client's Self, polarized protectors generally become willing to allow the Self to take over the job of caring for, protecting and repatriating the exile. Protectors are then freed from an onerous job and can find their preferred role in the internal family.

Post-unburdening: The three or four weeks following an unburdening are a window of time in which the physiological and emotional changes are consolidated.

Protector Check-in After Unburdening: Inviting protectors to try something new – allow the client's Self to be with (that is, heal) and protect the exile.

Retrieval: After being witnessed in whatever way it needs, an exiled part leaves the past (where it has continued to be, de facto, frozen in time) and comes into the present.

Self: The innate presence in each of us that brings balance and harmony along with certain nonjudgmental, transformative qualities (curiosity, caring, creativity, courage, calmness,

connectedness, clarity, compassion, presence, patience, persistence, perspective, playfulness) to our internal family. While parts can blend with (overwhelm and therefore obscure) the Self, the Self nevertheless continues to exist and is accessible as soon as parts separate (that is, unblend).

Self-energy: The perspectives and feelings that the Self brings to the relationship with parts.

Self-led: When an individual has the capacity to hear, understand, and be present with parts, acknowledging and appreciating the importance of their roles in the internal family system and with other people.

The Invitation for Exiles: After unburdening, the part can invite qualities of its own choosing to fill the space formerly occupied by the burden.

The Unburdening Process: Taken as a whole, the unburdening process includes witnessing, do-over, retrieval, unburdening, inviting new qualities and protector check-in.

Unblended (or ***differentiated***, or ***separated***): The state of being-with, in which no part (e.g. feeling, thought, sensation, belief) is overwhelming the Self. When unblended parts remain separate, present and accessible but are not vying to dominate, we have access to Self-qualities. This state of being unblended is often experienced as internal spaciousness.

Unburdening: The painful emotions, traumatic sensations and harsh beliefs of an exiled part are ceremonially released, often using imagery that involves one of the elements (light, earth, air, water, fire).

Witnessing: The process in which a part shows and/or tells the client's Self about its experiences until it feels understood, accepted, self-accepting and loved.

SEE FOR YOURSELF: PARADIGM SHIFT IN IFS

In the interest of anchoring the concepts of IFS, here is a preliminary illustration of assessment and diagnosis at the outset of an IFS therapy with a traumatized client.

On the phone before her first session, Serena offers some cursory details about her life and then firmly asserts that she doesn't want to spend a lot of time going over the past because her childhood was not the problem. She says she has tried talking about her childhood in therapy in the past but it didn't help. Mostly she is puzzled at the strong reaction she is having to her German boyfriend having broken up with her because he was leaving the United States.

SERENA: It's not like we were that serious. I just can't stop crying.

THERAPIST: Have you been in therapy before?

SERENA: Yes it was boring. I swore I would never do it again.

- *In this brief exchange the therapist learns a few important things about Serena, which she frames according to the IFS model.*
 - *Serena has a part who feels very sad but she has no idea why.*
 - *The relationship with her German boyfriend was either casual or a protective part is now minimizing its importance.*
 - *Something about therapy in the past was so unbearable that her protectors swore never to let her go back.*

When she comes in for the first session the therapist writes all this on a whiteboard so they can look at it together.

Serena's parts:

Can't stop crying

Puzzled about why Serena is crying

Relationship with boyfriend was not important

Will never go to therapy again!

Introducing the Concept of Communicating With Parts

THERAPIST: You've mentioned all these feelings and thoughts. I often find that when we focus on them internally and listen to them, we can learn important things about ourselves. Are you open to trying this? See which part needs your attention first.

SERENA: Why can't I stop crying?

Switching to Parts Language and Getting Permission to Proceed

THERAPIST: Good let's check on that. Ask if any of these other parts of you object to you helping the one who can't stop crying.

- *Asking other parts for permission to proceed is always wise once you choose a target part.*

SERENA: It's kind of funny but I'm hearing someone yelling, "I don't want to be here!"

Welcoming All Parts

THERAPIST: You did have that part who swore never to go to therapy again so that makes sense. Is it okay to hear more?

- *Inviting polarized parts (parts who may disagree) to participate helps forestall any urge they might have to sabotage the therapy and also provides crucial information.*

SERENA: I guess so.

THERAPIST: How do you feel toward the part who doesn't want to be here?

SERENA: Kinda curious about it.

- *This more open-hearted attitude signals that we can proceed.*

Remember the Target Part

THERAPIST: Okay, first let's tell the one who can't stop crying that we'll come back to it.

- *As with family therapy, we are polite and inclusive of all parts.*

SERENA: Seems like the crying is connected to the one who doesn't want to be here but I don't know how.

THERAPIST: Would you like to find out how?

- *Checking to see if she remains curious and will be available to hear this information.*

SERENA: Yes.

- *Serena has successfully shifted into being curious about and observing her inner experience without a lot of fear or judgment.*

Making Connections

THERAPIST: Ask the one who doesn't want to be here to tell you more.

SERENA: It's afraid I'll be overwhelmed.

- *Serena has a protective part who fears that giving the crying part attention will encourage it to take over, which would overwhelm her with negative emotion.*

THERAPIST: Do you understand this fear?

- *Again checking to see if Serena remains available (curious enough) to hear more about the crying part, who is in distress.*

SERENA: I was in a car crash when I was five. My mother died. But I don't remember her so I never think about it.

- *Serena is now beginning to be aware of her parts rather than being dissociated or overwhelmed. This 5-year-old part was exiled after her mother's death. Protectors (other parts) keep the 5-year-old out of her consciousness.*

THERAPIST: We can help the 5-year-old not take you over.

- *The therapist begins to reassure Serena's protectors that the 5-year-old can separate so that she can be safely helped.*



After this first meeting, the therapist is aware of the following: Serena has a traumatized part: a 5-year-old whose life was upended by the random violence of her mother's death in a car crash. Protective parts have kept this 5-year-old out of mind. And the part who removed Serena from therapy years ago does not feel safe about her returning to therapy now because of the danger of the distressed 5-year-old's overwhelming her emotionally.

Although the therapist knows all this, much remains unknown: what do Serena's internal and external systems believe about her mother's death? She might have parts – or actual external people – who hold her responsible. She might have beliefs about god, punishment, safety and fate. She might have parts who feel survivor guilt (the belief, for example, that surpassing her mother in happiness or longevity would be a betrayal). Or parts who feel separation guilt (for example, the belief that growing up and leaving her father would wound him). This initial assessment is just a beginning. There is much to learn and therapy itself will be the learning process.

THE GOALS OF IFS

Each step of this therapy has a goal. At every point the therapist helps the client to help the part. **First**, clients help their protective parts differentiate. **Second**, clients befriend their protective parts and get permission to help wounded parts. **Third**, clients form a positive relationship with wounded parts, witness their experiences and help them to let go of feeling states and beliefs that are extreme and damaging so they can heal. Accomplishing this milestone liberates protective parts, makes room for healed parts to reintegrate and reinstates the Self as leader of the inner system.

Two Categories of Protective Parts: Proactive and Reactive Proactive Parts

All protective parts are trying to exile the powerful negative feelings and beliefs of wounded parts with the aim of warding off more harm and keeping us safe. However protectors differ in that they are either proactive or reactive in response to emotional pain.

We call proactive parts “managers” because they try to manage our lives in ways that keep emotional pain out of consciousness. They often focus on motivating us to improve, work hard, be productive and be socially acceptable. At the extreme, however, these aims can devolve into tactics like perfectionism, intellectualizing, one-sided caretaking, obsessing about appearance, conflict avoidance at great personal cost and trying to control or please others.

Reactive Parts

We call reactive protectors “firefighters” because they try to distract from or stop emotional pain as fast as possible without consideration for consequences. These protectors view their actions the way we might view a life-saving medication with terrible side effects: If you need it, use it. Examples include bingeing, purging, addictions, numbing, dissociating and cutting as well as thoughts and behaviors related to suicide.

A CAVEAT ON PROACTIVE PROTECTORS

Although proactive protectors (managers) generally do seem managerial, any behavior that is used to prevent emotional pain is proactive. Consider, for example, that addiction and dissociation, which are generally reactive behaviors that serve to distract from intense negative feelings, can also be used to prevent us from having feelings at all. If a person goes from binge drinking to daily drinking, his reactive behavior has been recruited to the proactive role of preventing the feelings that drinking suppresses. If a person goes from dissociating in response to certain feelings to being numb and detached all the time, her reactive behavior has likewise been recruited to the proactive role of prevention. Extreme protectors are most likely to become proactive in this way in response to the looming threat of emotional overwhelm. Rather than trying to control or manage extreme protectors, in IFS we offer to resolve the underlying problem.

VULNERABLE PARTS: EXILED

When children feel shamed (often but not exclusively interpersonally) vulnerable young parts are particularly liable to develop overwhelmingly threatening beliefs like “I’m unlovable” and “I’m worthless.” Equally, when experience is terrifying and beyond our capacity to tolerate, our most vulnerable parts feel stripped of significance. Protectors step in to keep their toxic beliefs out of consciousness and, as a result, vulnerable parts end up permanently alone, forgotten and often trapped in the past. They long for help but when they push into consciousness with

negative feelings, beliefs, sensations and memories, protectors again experience them as a hazard. On the other hand, IFS therapists know that exiled parts are not their wounds. Clients come to therapy with exiles feeling miserable, but we know these parts will revert to their natural state of curiosity, creativity and playfulness once they are unburdened of trauma-derived beliefs. Their vitality and capacity for unself-conscious joy make a unique contribution to what Marsha Linehan calls “a life worth living” (1993).

DEFINING THE SELF

The Self is the core of psychic balance, the seat of consciousness and inner source of love. Everyone has a Self. Just as light can be both particle and wave, the Self can show up in the energy of certain feeling states (curiosity, calm, courage, compassion, love) or with the sense of an individual being present (Schwartz, 1995). But the Self is perhaps most simply introduced to clients as “the you who is not a part” (which we will use throughout this manual). Since parts who separate and get into relationship with the Self feel loved – and love provides an abiding counter to the sense of being unacceptable – the overarching goal of IFS is access to the Self. As a way of being, Self-energy helps us and our clients take a calm, curious, open stance toward inner experience. For some individuals this practice is spiritual in nature, for others it simply works.

ACCESSING THE SELF

Though extreme protective parts can block our access to it, the Self does not need to be cultivated or developed. Some meditation practices, most spiritual traditions, and some other models of psychotherapy feature a concept of core wisdom and balance similar to the Self, though using different language. Our goal in IFS is for both therapist and client to access Self-energy as a wave and to be in relationship with the Self as particle. As therapists, our practice is to be mindful of the needs of our reactive parts but to differentiate from them and be fully present with our clients.

Severe Trauma and The Self

In cases of severe trauma we think in terms of the “Self of the system,” which is to say the therapist and client are one system that has access to Self-energy through the Self of the therapist. Clients who experience dissociative identity disorder (DID), for example, may have

little or no access to the Self for months or years so the therapist must act as the Self of the therapeutic system. As inner attachments form and inner relationships are repaired, clients gain more access to their Self and the therapist can gradually move to a more supportive role. When the client has enough access to the Self and the baton passes from therapist to client, protectors (who are often young in age and are also traumatized) may need to be reassured that the therapist will not disappear.

When an Exile Meets the Self and Gets Mad

Often in trauma, when protectors first grant the client's Self access to the vulnerable part who has been exiled, it commonly responds with distrust or anger.

- “Where have you been?”
- “If you exist, why did I have to go through all that?”

When this kind of interaction occurs at the end of a session, we begin the next session by returning to the same part and same subject.

- Does this part want to say more to the client's Self about how it felt to be alone?
 - “What was it like to feel abandoned?”
 - “What is it like to wish this reunion had happened sooner?”

In response to these justified complaints of a part who feels abandoned, we take time to repair the relationship between it and the client's Self.

- “I apologize. You didn't deserve to be left alone. I'm so sorry I couldn't be here sooner. What has it been like for you?”
- “I'm here for you now. What do you need?”



ASSUMPTIONS IN IFS

1. All parts have good intentions, even those who misbehave. Therefore we begin therapy with a radical invitation: all parts are welcome.
2. Our psychic response to injury is predictable: when vulnerable parts are wounded other parts step into protective roles.
3. Protective parts behave in predictable ways, some of which look pathological.
4. A destabilized, disrupted inner system can become reintegrated and balanced once it is in relationship with the client's Self.
5. The Self is neither created nor cultivated and cannot be destroyed but is, rather, intrinsic and present from birth.
6. Every person has a Self, and the Self can be accessed for healing in every person.